

V11 ERGONOMICS PROGRAMMING | O (MAX : 3 PT)

Intent : Enhance well-being and comfort through comprehensive ergonomics programming.

Summary : This WELL feature requires projects to work with a certified ergonomist to implement comprehensive ergonomics programming, commit to on-going improvements to ergonomic design and provide ergonomic support for remote workers.

Issue : In 2016, musculoskeletal disorders (MSDs) ranked among the top drivers of global disability.^{1,2} MSDs are one of the most commonly reported causes of lost or restricted work time and also contribute to absenteeism and low productivity among employees.^{3,4} These losses amount to significant impacts for the economy at large and a business's bottom line.⁵ Ergonomic issues affect many sectors from schools to industrial settings and commercial offices, each with unique, contextual risks and considerations.⁶

Solutions : Ergonomic interventions aim to accommodate all individuals and increasingly adopt a holistic approach that tackles the design of the physical environment (e.g., adjustable furniture), the work itself (e.g., process, practices) and behavior (e.g., education, training).^{7,8} Ergonomic interventions have been shown to have a positive impact and a substantial return on investment (ROI). One study found that after implementing a macro-ergonomics intervention, claims over a 5-year period were reduced by 45% (200 fewer total claims) and researchers determined an ROI of 10:1 for the program. In this study, ROI calculations considered the compensation costs per claim, the number of preventive ergonomic evaluations performed and the annual cost of the program.⁹ Another study examining outcomes across 250 case studies, across a variety of sectors from healthcare (36) to offices (40), manufacturing facilities (87) and other industries, found generally positive results and noted that the payback period was generally less than one year.¹⁰

PART 1 IMPLEMENT AN ERGONOMICS PROGRAM (MAX : 1 PT)

For All Spaces:

1: Professional ergonomics support

The project or organization meets at least one of the following requirements:

- a. Engages with a qualified professional ergonomist (which may be a consultant, contractor or other third-party).
- b. Has at least one qualified professional ergonomist on staff whose responsibilities include ergonomic programming, as defined in their job description and/or performance expectations.

2: Ergonomics programming

The project or organization has an ergonomics program that includes the following:

- a. Annual consultation with key stakeholders (e.g., human resources, workplace wellness, occupational safety, leadership, employees) who are involved in the design, implementation and evaluation of the ergonomics program.
- b. A task analysis performed by a qualified professional ergonomist to identify the job-related tasks that are performed by occupants in the space.
- c. Accommodations for eligible employees to receive individual ergonomic assessments in the form of a self-assessment (e.g., reputable, third-party app), in-person assessment (e.g., at the workplace or home) or a virtual assessment (e.g., live video conference). Assessments are offered to employees at least annually and, as applicable, during or after the following events:
 1. Employee on-boarding.
 2. Substantial equipment changes (e.g., purchase of a new chair) or workstation redesign.
 3. Change in health status (e.g., injury, presentation of symptoms of musculoskeletal issues, visual strain).
 4. Change in work environment (e.g., transition to or from full-time remote work).
- d. Ergonomic trainings (e.g., workshop, seminar, classes) delivered by a qualified professional ergonomist to employees at least annually.

PART 2 COMMIT TO ERGONOMIC IMPROVEMENTS (MAX : 1 PT)

Note :

Projects may only achieve this part if Part 1 is also achieved.

For All Spaces:

Option 1: Informed Ergonomic Design

The following requirement is met:

- a. The project or organization describes how Part 1 informed design-decisions within Feature V02: Ergonomics Workstation Design and, as applicable, Feature V07: Active Furnishings.

OR

Option 2: Individual Ergonomic Needs

The following requirement is met:

- a. The project or organization demonstrates a commitment to addressing the individual ergonomic needs of employees identified through individual ergonomics assessments.
- b. The timeline for delivery of solutions is communicated to employees.

PART 3 B SUPPORT REMOTE WORK ERGONOMICS (MAX : 1 PT)

Note :

Projects may only achieve this part if Part 1 is also achieved.

For All Spaces:

For organizations with employees who regularly work remotely, the organization tailors the ergonomic program addressed in Part 1 in the following ways:

- a. For individual assessments, accommodations are made to include remote workers (e.g., virtual assessments).
- b. The organization provides ergonomic equipment to support the individual needs of remote workers through direct-purchases, reimbursement or subsidies.

Note : This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

REFERENCES

1. Institute for Health Metrics and Evaluation (IHME). GBD Compare. 2015. <http://vizhub.healthdata.org/gbd-compare>.
2. Agarwal S, Steinmaus C, Harris-Adamson C. Sit-stand workstations and impact on low back discomfort: a systematic review and meta-analysis. *Ergonomics*. 2018. doi:10.1080/00140139.2017.1402960
3. Bevan S. Economic impact of musculoskeletal disorders (MSDs) on work in Europe. *Best Pract Res Clin Rheumatol*. 2015;29(3):356-373. doi:10.1016/j.berh.2015.08.002
4. Occupational Safety and Health Administration. *Ergonomics - Overview*. <https://www.osha.gov/SLTC/ergonomics/>. Accessed October 31, 2017.
5. Centers for Disease Control and Prevention. *Work-Related Musculoskeletal Disorders & Ergonomics*.
6. Centers for Disease Control and Prevention, The National Institute for Occupational Safety and Health. *Elements of Ergonomic Programs*. 1997. <https://www.cdc.gov/niosh/topics/ergonomics/ergoprimer/default.html>.
7. Safe Work Australia. *Principles of Good Work Design*. <https://www.safeworkaustralia.gov.au/system/files/documents/1702/good-work-design-handbook.pdf>.
8. Hedge A. Ergonomic Workplace Design for Health, Wellness, and Productivity. In: *Ergonomic Workplace Design for Health, Wellness, and Productivity*. ; 2016:1-443. doi:10.1201/9781315374000
9. Heller-ono A. A Prospective Study of a Macroergonomics Process over Five Years Demonstrates Significant Prevention of Workers' Compensation Claims Resulting in Projected Savings. 2009:1-4. <http://worksitesinternational.com/pdf/ODAM-NES-2014-ARH-paper-Final-Submission.pdf>.
10. Goggins RW, Spielholz P, Nothstein GL. Estimating the effectiveness of ergonomics interventions through case studies: Implications for predictive cost-benefit analysis. *J Safety Res*. 2008;39(3):339-344. doi:10.1016/J.JSR.2007.12.006